

ED door-to-doctor project charter

The dense charter Marcus emailed Priya to read before the kickoff, laid out field by field.

The project charter (verbatim)

PROJECT CHARTER - CONFIDENTIAL / INTERNAL

Project title: Reduce Emergency Department Door-to-Doctor Time

Sponsor: VP, Clinical Operations

Process owner: ED Service Line Director

Green Belt lead: Marcus T.

Project team: ED charge nurses, registration lead, lab liaison, white belt (operations)

1. Problem statement

Across the system's three hospital campuses, the median door-to-doctor interval (DTD: elapsed time from patient arrival/registration to first evaluation by a physician or advanced-practice provider) currently measures 52 minutes against an internal benchmark of 30 minutes. Prolonged DTD correlates with elevated left-without-being-seen (LWBS) rates and reduced HCAHPS (patient-experience survey) scores. Estimated annualized impact of LWBS attrition and throughput loss is approximately \$1.4M.

2. Goal statement

Reduce median DTD from 52 minutes to 30 minutes (a 42% reduction) across all three campuses within two fiscal quarters, without increasing downstream length-of-stay.

3. Scope

In scope: the patient pathway from ED arrival through first provider evaluation, all acuity levels, all three campuses, 24/7. Out of scope: inpatient boarding processes upstream of bed assignment, EMS pre-arrival protocols, and the urgent-care network.

4. Critical-to-Quality characteristics (CTQs)

Time from arrival to triage completion.

Time from triage to bed assignment.

Time from bed assignment to provider evaluation.

5. High-level process (SIPOC summary)

Suppliers: patients, EMS, registration. Inputs: patient arrival, demographic/insurance data, acuity assessment.

Process: arrival to triage to registration to bed wait to nurse assessment to provider evaluation. Outputs: evaluated patient, disposition decision. Customers: patients, downstream inpatient units.

6. Baseline and target

Baseline median DTD: 52 min. Target: 30 min. Measurement window: trailing 90 days, all campuses pooled.

7. Estimated financial impact

Approximately \$1.4M annualized (LWBS recovery and throughput). (Figure pending Finance confirmation.)

8. Timeline and risks

Two-quarter timeline aligned to DMAIC. Key risks: seasonal volume surges, inpatient boarding constraints outside ED control, and registration staffing variation across campuses.

This is the demo charter for the C49 decode (the ED door-to-doctor project). It is the document the AI summarizes, defines roles from, and runs the 42% reduction check against in the lecture.